

HOSPITAL APPROVED CLINICAL PRACTICE GUIDELINE: RESPIRATORY INFECTIONS
DOCUMENT REF: HCG-RESP-2026-04
APPROVAL COMMITTEE: Clinical Quality & Safety Board

SECTION 1: COMMUNITY-ACQUIRED PNEUMONIA (CAP)

1.1 DIAGNOSTIC CRITERIA:

Diagnosis requires the presence of acute respiratory symptoms (cough, fever, dyspnea, pleuritic chest pain)

1.2 RISK STRATIFICATION (CURB-65 SCORE):

- Confusion (mental status)
- Urea > 7 mmol/L (BUN > 19 mg/dL)
- Respiratory rate $\geq$ 30 breaths/min
- Blood pressure (Systolic < 90 mmHg or Diastolic $\leq$ 60 mmHg)
- Age $\geq$ 65 years

Score 0-1: Low risk (Outpatient management suitable).

Score 2: Moderate risk (Consider short inpatient admission).

Score $\geq$ 3: High risk (Urgent inpatient admission, assess for ICU care).

1.3 EMPIRICAL ANTIMICROBIAL THERAPY GUIDELINES:

- Outpatient / Low Risk: Amoxicillin 1g TID orally or Doxycycline 100mg BID.
- Inpatient Non-Severe CAP: Intravenous Beta-lactam (Ceftriaxone 1g-2g daily or Ampicillin-Sulbactam 1.5g-3g daily).
- Severe / ICU Admission: Ceftriaxone 2g daily PLUS Azithromycin 500mg IV daily, with MRSA coverage (Clindamycin or Vancomycin).

1.4 MONITORING AND DISCHARGE CRITERIA:

Afebrile for at least 48 hours, normalized heart rate (<100 bpm), respiratory rate (<24 bpm), oxygen saturation (>92%)